

Diabetes - High Blood Sugar - Male (Child)

Algorithm 1077 | Child (0-17 years) | Gender at birth: Male

Definition

- Hyperglycemia (high blood sugar) and diabetic ketoacidosis risk assessment decomposed from NHS.UK's published when-to-get-help guidance.
- Child patient.
- Male.

Clinical background

- Hyperglycaemia mainly affects people with diabetes and can become serious if untreated.
- Common symptoms include thirst, frequent urination, weakness or tiredness, blurred vision, and weight loss.
- Vomiting, abdominal pain, rapid breathing, drowsiness, fruity breath, confusion, or high ketones can indicate a life-threatening complication.

Initial assessment

1. [OPEN_TEXT] What is the person's exact age and sex, are they pregnant, what glucose and ketone readings/units are shown, what symptoms are present, and what does their prescribed sick-day or pump plan say?
2. [YES_NO] Has a ketone level been checked (blood or urine)?
3. [YES_NO] Has diabetes medicine been taken as prescribed today?
4. [CAREGIVER_OBSERVATION] What change has the parent or caregiver observed in behaviour, alertness, eating or drinking, sleep, school or play, and normal activity?

Triage and redirect questions

1. Redirect → Diabetes - Low Blood Sugar

Is the measured glucose below 4 mmol/L, or are shaking, sweating, hunger, or confusion suggesting hypoglycaemia?

A low reading or hypo symptom pattern belongs in the low-blood-sugar pathway and needs prompt treatment.

1. Disposition level 100

Along with high blood sugar, is there nausea/vomiting or stomach pain, faster-than-usual breathing or heart rate, drowsiness, breath with a fruity ('pear drop') smell, confusion or difficulty concentrating, or a high ketone level?

NHS.UK high blood sugar guidance lists these as call-999/A&E criteria for possible diabetic ketoacidosis (DKA). Do not drive to A&E - call an ambulance.

1. Disposition level 50

Has blood sugar remained high or symptoms continued despite trying to lower it, or is this high blood sugar in someone not previously diagnosed with diabetes?

NHS.UK guidance recommends contacting a diabetes care team or GP for persistent high readings or a new, undiagnosed presentation.

Advice

Qatar emergency hyperglycaemia/DKA response

- Call Qatar emergency services on 999 and do not drive for vomiting, abdominal pain, deep/rapid breathing, fruity breath, dehydration, confusion/drowsiness, ketones above the person's emergency-plan threshold, or rapid deterioration. Follow only the person's prescribed sick-day/pump plan; do not improvise extra insulin. Do not force fluids when vomiting, drowsy, or unsafe to swallow, and do not delay 999 for another glucose or ketone check. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening drowsiness or confusion; vomiting, abnormal breathing, or dehydration; rising ketones or failure of the personal plan.

Routine high blood sugar follow-up

- Take diabetes medicine as prescribed, avoid sugary/starchy foods, manage stress, stay active, and follow sick-day guidance from the diabetes care team.
- Call Back If: nausea, vomiting, or confusion develops; breath develops a fruity smell.

First aid

- Call Qatar emergency services on 999 and do not drive for vomiting, abdominal pain, deep/rapid breathing, fruity breath, dehydration, confusion/drowsiness, ketones above the person's emergency-plan threshold, or rapid deterioration. Follow only the person's prescribed sick-day/pump plan; do not improvise extra insulin. Do not force fluids when vomiting, drowsy, or unsafe to swallow, and do not delay 999 for another glucose or ketone check. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "High blood sugar (hyperglycaemia)", <https://www.nhs.uk/conditions/high-blood-sugar-hyperglycaemia/> (page last reviewed 17 March 2026)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from the cited NHS.UK pages under the Open Government Licence v3.0; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect and is not a disposition row. Local disposition mappings and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: 38f9793396832f27c9ccd0783b80d5c74a89ddc47912def6fce03609d6ce452b | Status: BLOCKED_BY_RESEARCH_GAP